Decisions, Tasks and Open Questions

A short guide to putting your husband's care in order.

By now the list is long: the rehab's question, the pharmacy's questions, the appointments, the things your daughter wrote down. It reads as one list, and everything on it feels like the same kind of thing.

It isn't. Some items are decisions, some are tasks and some are open questions, and each one needs something different from you. This guide explains the difference and why the order matters. It explains. It doesn't put your list in order for you.

Start with the dates you were actually given

Urgency comes from a date, and a date comes from a person. A day the rehab's admissions office gave you is one thing. A day the family has started working to, without anyone having said it, is another.

So each date gets a name beside it. The Family Caregiver Alliance gives this advice for calls to agencies, and it works here too: write down everything you're told, with the date, who you spoke with and any agreed-upon decisions. [1]

For the discharge itself, the Alliance notes that only a physician can authorize a patient's release from the hospital, while the planning around it may be done by a discharge planner, a social worker, a nurse or another hospital representative. Those are the people to ask about that date. [2]

- Where there's no date, write “no date given.” Don't supply one.
- What you don't know yet stays on the page as a question. It doesn't get dropped because nobody could answer it on the day.

A decision, a task and an open question are different things

A decision is a choice somebody has to make: what answer to give the rehab. A task is something somebody has to do: pick up the prescriptions, return the call. An open question is something to find out: what the pharmacy needs to know, and who has the answer.

Each needs something different. A decision needs the right people and the facts it depends on. A task needs someone willing to do it. A question needs to reach the person who can answer it.

A worry isn't any of the three until you turn it into one. “I'm worried about his medicines” becomes a question for the pharmacist: “Which list is the current one?”

Medicare's discharge checklist tells caregivers to write down their questions about the discharge instructions and discuss them with the staff. On your page, a question that belongs to a doctor, a nurse or a pharmacist stays marked with whose it is until that person has answered it. [3]

What has to come before what

Some items can't sensibly be settled until another one is. The answer to the rehab may be waiting on something only his doctor can tell you. That's a dependency. Write the two together: the item that waits, and the item it waits on.

The Family Caregiver Alliance says something similar about family meetings: until the issues have been explored, it's important not to try to solve the problems, though recording them in a list as they come up will be useful. [4]

Some items aren't for doing this week at all. They're for keeping an eye on and writing down. If you can't tell which kind an item is, that's a question for the professional it belongs to.

And every task needs someone with room to do it. AARP's planning guide says caring for a family member can be too big a job for one person, and suggests asking team members which tasks they'd prefer. MedlinePlus puts it this way: make a list of ways others can help, and let helpers choose what they'd like to do. [5] [6]

The hospital needs to know your limits too. The Family Caregiver Alliance says discharge planners should discuss with you your willingness and ability to provide care, and that it is extremely important to tell them about the other obligations on your time. [2]

One page everyone can see

What's been agreed needs to be written where everyone involved can read it. The Family Caregiver Alliance suggests a written summary of what each person has agreed to, and making sure each person understands what he or she has agreed to do. [4]

For the tasks that matter most, the page gets a second name: someone who steps in if the first person can't. And each decision that's still open gets a day when the family will look at it again.

It isn't done once. The Alliance says family meetings need to take place regularly, or at least when the caregiving situation or family members' own lives change, and that agreements can be made for a limited time to see whether they work. [4]

Your husband belongs in all of it. AARP's guide says a plan should never be made without the participation, knowledge and consent of the person receiving the care. [5]

What this means for your family

- The long list becomes three shorter ones: decisions, tasks and open questions.
- Each date gets a name beside it. A date nobody gave you is a question to ask.
- Questions for professionals stay on the page until the right person has answered them.
- The work gets shared by asking and offering. Nobody's name goes on a task they haven't agreed to.
- Putting the decisions in their final order comes after this, on the whole picture of your husband's situation and with a professional's help.

What this guide doesn't do

It doesn't put your family's decisions in order, say what your husband needs, or tell you what to answer the rehab. The advice with a number beside it comes from the sources below. The three kinds of item, and the habit of writing down what waits on what, are the Family Care Roadmap's own.

Ready for the next step?

You can now tell a decision from a task from an open question, and you know why the order matters. The care interview gives that order something to stand on: it walks you through your husband's situation area by area, so nothing important is left out of it, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Family Caregiver Alliance, Caregiving at Home: A Guide to Community Resources
2. Family Caregiver Alliance, Hospital Discharge Planning: A Guide for Families and Caregivers
3. Centers for Medicare & Medicaid Services, Your Discharge Planning Checklist (CMS Product No. 11376)
4. Family Caregiver Alliance, Holding a Family Meeting
5. AARP, Prepare to Care: A Planning Guide for Families
6. MedlinePlus (National Library of Medicine, NIH), Caregiver Health

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